

Clinical Conditions Handbook (Verbatim-to-Natural-Language)

Generated from clinicalguide-ai dataset on 2026-05-04

This handbook presents each condition using the original dataset content wherever possible, arranged into readable natural-language management guidance. It is intended for RAG ingestion and clinician-facing support.

Clinical safety note: Use this as decision support only. Always apply local protocols, supervision, and bedside clinical judgment.

1. hotness of body

For hotness of body, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; Duration; Pre- Treatment; Age; Signs & symptoms; Investigations; Results; Additional tests(sample collected in out patient); Diagnosis; Plan; SUPPORTIVE MANAGEMENT; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

2. Elevated BPs in Pregnancy

For Elevated BPs in Pregnancy, the dataset emphasizes evaluating the following clinical parameters: Presentation; Pregnant; Signs & symptoms; Examination Findings; Investigations; Results; Additional tests(sample collected in out patient); Diagnosis; Plan; SUPPORTIVE MANAGEMENT; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

3. EPIGASTRIC PAIN

For EPIGASTRIC PAIN, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; Signs & symptoms; Supportive Management; Investigations; Results; ADDITIONAL TESTS; Diagnosis; Plan; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed; EPIGASTRIC PAIN; bloody Vomitus WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

4. PV BLEEDING

For PV BLEEDING, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; Duration; Pre- Treatment; Pregnant; Signs & symptoms; Examination Findings; Investigations; Results; Additional tests(sample collected in out patient); Diagnosis; Plan; SUPPORTIVE MANAGEMENT; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

5. BURNS

For BURNS, the dataset emphasizes evaluating the following clinical parameters: Presentation; ADMISSION CRITERIA IRRESPECTIVE OF SIGNS & SYMPTOMS; Age; TBSA; Thickness; Supportive Management; Investigations; Diagnosis; Plan; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed; PATIENT WITH BURNS.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

6. CHEST TRAUMA

For CHEST TRAUMA, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; Signs & symptoms; PERCUSSION NOTE; Supportive Management; Investigations; Additional tests(sample collected in out patient); Diagnosis; Plan; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed; Trauma to the Chest + Difficulty in breathing.; WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

7. HEAD INJURY

For HEAD INJURY, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; GCS; Signs & symptoms; Supportive Management; Investigations; Additional tests(sample collected in out patient); Diagnosis; Plan; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed; Trauma to the head; GCS > 13.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

8. ABDOMINAL INJURY

For ABDOMINAL INJURY, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; Penetrating; Signs & symptoms; Supportive Management; Investigations; Additional tests(sample collected in out patient); Diagnosis; Plan; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed; Abdominal Injury; NO.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

9. Sheet2

For Sheet2, the dataset emphasizes evaluating the following clinical parameters: RISK FACTORS FOR PPH; PRENATAL; ANTEPARTUM; DURING ADMISSION; INTRAPARTUM; MEDIUM; HIGH RISK; Suspected previa/increta/percreta; Placent previa; prior C/S (MEDIUM); placenta previa; Chorioamnionitis; NEW Active bleeding; BMI> 50; placenta accreta; Uterine surgery; suspected Accreta/Percreta; prolonged oxytocin > 24 hours; 2/> medium risk factors(admission OR intrapartum); prior classical C/S.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

10. WHEEZE

For WHEEZE, the dataset emphasizes evaluating the following clinical parameters: Presenting complaints; Signs & symptoms; Investigations; Diagnosis; Plan; SUPPORTIVE MANAGEMENT; Treatment; Brand Name; CASH IP(BRAND); Consultation; Laboratory; Imaging; Pharmacy Adult; Pharmacy Paeds; Total Cost Adult; Total Cost Paed; Wheezing with Difficulty in Breathing; WITH ANY OF: While walking, Talks in

Alertness, Can lie down, Sentences May be agitated, Increased Respiratory rate, no use of accessory muscles, wheeze often only end expiratory, Pulse Rate <100, Pulsus paradoxus, Absent/ <10 mm Hg; F
MILD ASTHMATIC ATTACK.

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

11. HYPONATREMIA

For HYPONATREMIA, the dataset emphasizes evaluating the following clinical parameters: HYPONATRAEMIA TREATMENT.; (Serum Na < 130 mmol/L); signs and symptoms; nausea, malaise, lethargy, decreased level of consciousness, headache, if severe (seizures and coma), overt neurologic symptoms (<115 mEq/L); CLASSIFICATION; 1. Hypovolemic- decrease in total body water with greater decrease in total body sodium; 2. Euvolemic- normal body sodium with increase in total body water; 3. Hypervolemic- increase in total body sodium with increase in total body water; LAB WORKUPS; MANAGEMENT; In the event of a seizure, coma or suspected cerebral herniation as a result of hyponatremia, IV 3% hypertonic saline should be administered as soon as possible according to the following guide:; a. Administer 3% hypertonic saline 100-150cc IV over 5-10min; b. Reassess patient; c. If the patient does not improve clinically after the first bolus, repeat a second bolus of hypertonic saline.; d. STOP ALL FLUIDS after the second bolus of hypertonic saline to avoid raising the serum sodium any further; What if hypertonic saline is not readily available? Administer one amp of Sodium Bicarbonate over 5min.; Defend the Intravascular Volume; In order to defend the intravascular volume a determination an assessment of the patient's volume status must occur. Is the patient hypovolemic, euvolemic or hypervolemic?; Although volume status is difficult to assess with any accuracy at the bedside, a clinical assessment with attention to the patient's history, heart rate, blood pressure, JVP, the presence of pedal and sacral edema.; Make a rough determination of whether the patient is significantly hypovolemic (requiring fluid resuscitation) or significantly hypervolemic (requiring fluid restriction +/- diuretics)..

If no explicit drug/action rows are present, continue supportive care, monitor progression, and use pathway criteria above to escalate, continue, or de-escalate treatment.

For follow-up queries, keep the same condition context, reassess severity and red flags, and update the next management step only after checking whether the patient now meets a different branch criterion.

Appendix: RAG-friendly response contract

Each answer should include: condition identified, branch selected with reason, one next safe management step, and one clarification question only when branch selection is uncertain.

Suggested JSON schema: assistant_reply, next_actions, clarification_questions, insufficient_evidence.